

8-12 WEEKS

BEGINNER

TESTOSTERONE ONLY

COMPLETE PCT

# BEGINNER ANABOLIC STEROID CYCLE COMPLETE GUIDE

Testosterone-Only Foundation · Safe Protocols · Full PCT · Side Effect Management

*Evidence-based · Minimal risk · Maximum results for first cycles*

ELITE FITNESS GUIDE SERIES

Research-Based · Professional Edition · 2024

■ **MEDICAL DISCLAIMER:** For educational purposes only. Anabolic steroids are controlled substances. Always consult a licensed physician.

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## ■ 1. THE GOLDEN RULE — WHY TESTOSTERONE ONLY

Every first cycle should be **Testosterone only**. This is not a suggestion — it is the most important rule in anabolic steroid use for beginners. Here is the science and logic behind it:

■ *Testosterone was the first anabolic steroid synthesized (1935, Ruzicka & Butenandt). Every other anabolic steroid is a structural modification of testosterone. Your body naturally produces testosterone — making it the most understood, most researched, and most predictable compound available.*

- **Isolation principle:** With one compound, any side effect you experience is immediately identifiable. Add multiple compounds and you cannot know which caused which reaction
- **Maximum response:** Beginners respond more dramatically to first cycles than advanced users. You will gain 15–25 lbs on testosterone alone — more compounds do not improve beginner results significantly
- **Reversibility:** A testosterone-only cycle with proper PCT is the most recoverable AAS protocol. More complex stacks suppress the HPT axis more severely
- **Establish your AI dose:** Every person aromatizes differently. Testosterone only lets you find your exact AI requirement before adding more variables
- **Baseline biomarkers:** First cycle establishes how your liver, lipids, and hormones respond — critical data for future protocols

## ■ 2. HOW ANABOLIC STEROIDS WORK — THE SCIENCE

Understanding the mechanism helps you make informed decisions, recognize side effects early, and manage your cycle intelligently.

### The Androgen Receptor (AR) Pathway:

- Testosterone enters the cell and binds to the androgen receptor (AR) in the cytoplasm
- The testosterone-AR complex translocates to the nucleus and binds to Androgen Response Elements (AREs) on DNA
- This activates transcription of muscle-specific genes: myosin heavy chain, actin, IGF-1, and more
- Result: increased protein synthesis, nitrogen retention, and satellite cell activation
- Also reduces catabolic gene expression — your body degrades muscle protein more slowly

### Why Supraphysiological Doses Build More Muscle:

Natural testosterone levels peak at 400–800 ng/dL. At 500 mg/week of Test E, blood levels reach approximately 2,000–3,500 ng/dL. This 3–5× supraphysiological level saturates and overstimulates androgen receptors, driving protein synthesis at rates impossible to achieve naturally.

■ *The landmark Bhasin study (1996, NEJM) demonstrated men given 600mg testosterone/week for 10 weeks gained 13 lbs of lean mass with NO exercise — compared to 4 lbs in natural trainees WITH exercise. This definitively established the dose-response relationship between testosterone and muscle mass.*

## ■ 3. TESTOSTERONE ENANTHATE/CYPIONATE — COMPLETE BREAKDOWN

| Property             | Testosterone Enanthate    | Testosterone Cypionate     |
|----------------------|---------------------------|----------------------------|
| Ester                | Enanthic acid (C7)        | Cypionate acid (C8)        |
| Half-Life            | 7–8 days                  | 8–10 days                  |
| Injection Frequency  | 2× weekly (e.g., Mon/Thu) | 2× weekly or once weekly   |
| Time to Steady State | ~3.5 weeks                | ~4 weeks                   |
| Common Brands        | Generic, Testoviron       | Depo-Testosterone, generic |
| Interchangeable?     | Yes — nearly identical    | Yes — nearly identical     |

### Dosage by Experience Level:

| Experience                        | Weekly Dose | Expected Lean Gain | Notes                                                       |
|-----------------------------------|-------------|--------------------|-------------------------------------------------------------|
| True beginner (1st ever cycle)    | 300 mg/wk   | 12–15 lbs          | Minimal side effects; excellent results; establish response |
| Intermediate beginner (2nd cycle) | 400 mg/wk   | 15–20 lbs          | Balanced risk/reward; well-tolerated by most                |
| Advanced beginner (3rd+ cycle)    | 500 mg/wk   | 18–25 lbs          | Maximum beginner benefit; manage AI carefully               |

## ■ 4. OPTIONAL KICK-STARTER: DIANABOL

Testosterone Enanthate/Cypionate takes 3–4 weeks to reach therapeutic blood levels (due to the long ester). Some athletes add **Dianabol** for the first 4–6 weeks to feel immediate effects while the long ester builds up. This is optional and **not recommended for a true first cycle**.

■■ **IMPORTANT:** Do not add Dianabol to your **FIRST** ever cycle. Run testosterone only first to establish your response to AAS before adding another compound. Dianabol causes significant liver stress and requires additional supplements.

- Dose if added: 20–30 mg/day split into 2–3 doses with meals
- Duration: Weeks 1–6 **ONLY** (oral AAS should not exceed 6–8 weeks continuously)
- Expect rapid weight gain (5–12 lbs in 3 weeks) — mix of muscle, glycogen, and water
- Water weight will decrease when Dbol is discontinued — this is normal and expected
- Mandatory additions with Dbol: TUDCA 500mg/day + increase AI dose due to higher aromatization

■ 5. PROTOCOL SELECTION — 8 vs 10 vs 12 WEEKS

| Protocol              | Stack                                   | PCT Start                       | Best For                                                     |
|-----------------------|-----------------------------------------|---------------------------------|--------------------------------------------------------------|
| 8-Week (Conservative) | 300–400mg Test E, optional Dbol wks 1–4 | 14–16 days after last injection | Absolute beginners; minimal suppression; fastest recovery    |
| 10-Week (Recommended) | 400–500mg Test E, optional Dbol wks 1–6 | 14–16 days after last injection | Best balance of results vs recovery; recommended for most    |
| 12-Week (Extended)    | 400–500mg Test E, optional Dbol wks 1–6 | 14–18 days after last injection | Maximum gains; longer recovery needed; experienced beginners |

| Weeks | Test E/C (2×/wk)              | Dbol (optional)     | Arimidex (EOD)        | Notes                                                |
|-------|-------------------------------|---------------------|-----------------------|------------------------------------------------------|
| 1–2   | 150–250mg Mon + 150–250mg Thu | 20mg/day with meals | 0.5mg every other day | Start AI from day 1; assess tolerance                |
| 3–4   | 150–250mg Mon + 150–250mg Thu | 30mg/day with meals | 0.5mg every other day | Blood work optional at Wk 4; note any symptoms       |
| 5–6   | 150–250mg Mon + 150–250mg Thu | 30mg/day with meals | 0.5mg every other day | Stop Dbol end of Wk 6; liver enzymes normalise       |
| 7–10  | 150–250mg Mon + 150–250mg Thu | — stopped           | 0.5mg every other day | Core training weeks; prioritise progressive overload |
| 11–12 | 150–250mg Mon + 150–250mg Thu | —                   | 0.5mg every other day | Final weeks; prepare nutrition + PCT supplies        |

Day-Wise Weekly Dosing Schedule (Testosterone Twice-Weekly Protocol):

| Day       | Test E/C Injection           | AI (Arimidex)          | Oral Dbol (if using) | Notes                                        |
|-----------|------------------------------|------------------------|----------------------|----------------------------------------------|
| Monday    | YES — 150–250mg (Pin 1 of 2) | 0.5mg (take with meal) | 15mg with breakfast  | Inject glute or quad; rotate sites           |
| Tuesday   | —                            | —                      | 15mg with breakfast  | Rest or cardio day                           |
| Wednesday | —                            | 0.5mg (EOD from Mon)   | 15mg with breakfast  | AI dose falls here if EOD                    |
| Thursday  | YES — 150–250mg (Pin 2 of 2) | 0.5mg (take with meal) | 15mg with breakfast  | Second injection; maintain 3–4 day gap       |
| Friday    | —                            | —                      | 15mg with breakfast  | Training day — strength should be elevated   |
| Saturday  | —                            | 0.5mg (EOD from Thu)   | 15mg with breakfast  | AI dose; active recovery                     |
| Sunday    | —                            | —                      | —                    | Rest day; prep food and supplements for week |

Tip: Set phone reminders for injection days (Mon + Thu) and AI days (Mon, Wed, Fri, Sun for EOD) to avoid missed doses.

## ■ 6. AROMATASE INHIBITOR — COMPLETE GUIDE

**Aromatization** is the conversion of testosterone to estradiol (E2) by the enzyme aromatase (CYP19A1), found primarily in fat cells, liver, brain, and muscle. At supraphysiological testosterone doses, aromatization increases proportionally — making AI use necessary for most men.

■ Approximately 0.3% of testosterone is converted to estradiol in healthy males. At 500mg/week testosterone, aromatization can produce 3–5× normal estradiol levels without AI intervention. Individual aromatization rate varies significantly based on body fat percentage and genetic factors.

### Signs of HIGH estrogen (over-aromatization):

- Gynecomastia — puffy, tender, or itchy nipples (glandular tissue forming under nipple)
- Water retention — smooth, bloated appearance; elevated blood pressure
- Mood instability — emotional, anxious, irritable; similar to PMS symptoms
- Erection difficulty — ED despite high testosterone is often caused by high E2
- Fatigue and sluggishness

### Signs of LOW estrogen (over-suppression with AI):

- Joint pain and cracking — estrogen lubricates joints; crash causes intense joint discomfort
- Very low libido and erectile dysfunction — estrogen is pro-sexual for men in optimal range
- Depression, anxiety, cognitive fog
- Poor workout pumps — estrogen helps glycogen storage and muscle fullness

| AI Option                            | Mechanism                                      | Starting Dose                | Adjustment                                                        |
|--------------------------------------|------------------------------------------------|------------------------------|-------------------------------------------------------------------|
| Arimidex (Anastrozole) — Recommended | Reversible competitive inhibitor of aromatase  | 0.5 mg EOD                   | Increase to 0.5 mg/day if E2 high; decrease to 0.25 mg EOD if low |
| Aromasin (Exemestane)                | Suicidal/irreversible aromatase inactivator    | 12.5 mg EOD                  | 12.5 mg/day if E2 high; 12.5 mg E3D if low                        |
| Letrozole (Letro)                    | Most potent AI — use only for gyno emergencies | 2.5 mg/day (short-term only) | NOT for routine cycle management — can crash E2 rapidly           |

## ■ 7. SIDE EFFECTS — RECOGNITION & MANAGEMENT

| Side Effect     | Probability       | Early Signs                                            | Intervention                                                                                          |
|-----------------|-------------------|--------------------------------------------------------|-------------------------------------------------------------------------------------------------------|
| Acne            | High (70–80%)     | Breakouts on back, shoulders, chest                    | Salicylic acid wash daily; benzoyl peroxide topical; reduce androgens; Accutane post-cycle for severe |
| Hair Loss (MPB) | Genetic (20–40%)  | Increased shedding; hairline recession                 | Topical Finasteride 0.025% solution; Nizoral 2% shampoo 2×/wk; accept genetic risk                    |
| Water Retention | High (60–80%)     | Puffiness in face/extremities; weight gain >1.5 lbs/wk | Reduce AI dose if caused by low E2; low sodium diet; adequate hydration (counterintuitive but helps)  |
| Gynecomastia    | Moderate (20–30%) | Puffy/sensitive nipples                                | Nolvadex 20mg/day immediately; add/increase AI                                                        |
| Elevated BP     | Moderate (30–50%) | BP >130/85 consistently                                | Monitor weekly; reduce sodium; add Hawthorn Berry; consult doctor if >140/90                          |
| Mood Changes    | Moderate (20–40%) | Increased irritability or aggression                   | Usually dose-dependent; communicate with partner/family; lower dose if severe                         |

## ■ 8. POST CYCLE THERAPY — THE MOST CRITICAL PHASE

PCT is often neglected or done poorly — this is the most common mistake in AAS use. **Failing PCT leads to prolonged low testosterone**, which causes loss of all cycle gains, depression, lethargy, sexual dysfunction, and potentially permanent hypogonadism in extreme cases.

### PCT Start Timing (for Testosterone Enanthate/Cypionate):

Wait **14–18 days** after your last injection before starting SERMs. The long ester must clear before SERMs can work — starting too early means the exogenous testosterone still suppresses your HPT axis while SERMs try to restart it.

| Compound            | Weeks 1–2 | Weeks 3–4 | Notes                                                                                                                                           |
|---------------------|-----------|-----------|-------------------------------------------------------------------------------------------------------------------------------------------------|
| Clomid (Clomiphene) | 50 mg/day | 50 mg/day | Primary LH/FSH stimulator — most powerful SERM for HPT restart. May cause visual disturbances (blurry vision) — stop immediately if this occurs |

| Compound             | Weeks 1–2                     | Weeks 3–4         | Notes                                                                                                                     |
|----------------------|-------------------------------|-------------------|---------------------------------------------------------------------------------------------------------------------------|
| Nolvadex (Tamoxifen) | 20 mg/day                     | 20 mg/day         | Anti-estrogenic SERM — synergistic with Clomid; also protects breast tissue                                               |
| HCG (optional)       | 500 IU E3D (2 wks BEFORE PCT) | Stop at PCT start | Restores testicular size and sensitivity; start 2 weeks after last injection, run for 2 weeks, then begin Clomid/Nolvadex |

■ *Post-PCT blood work target: Total Testosterone >400 ng/dL, LH >2 mIU/mL, FSH >2 mIU/mL. If values remain suppressed 8 weeks post-PCT, consult an endocrinologist regarding TRT evaluation.*

## ■ 9–12. BLOOD WORK, TRAINING, NUTRITION & SUPPLEMENTS

### Minimum Blood Work Protocol:

| When                      | Tests                                                                              |
|---------------------------|------------------------------------------------------------------------------------|
| Pre-cycle (baseline)      | CBC, CMP (liver/kidney), Lipids, Total T, Free T, LH, FSH, E2, Prolactin, TSH, PSA |
| Week 4 on-cycle           | E2, Hematocrit, ALT/AST                                                            |
| End of cycle (Week 10–12) | Full panel                                                                         |
| PCT Week 4                | Total T, LH, FSH, E2                                                               |
| 8 wks post-PCT            | Full panel — confirm recovery                                                      |

**Training Split:** Push/Pull/Legs 5–6 days/week. Focus on compound movements (squat, deadlift, bench, row, OHP). Progressive overload every session. Sleep 8+ hours.

**Nutrition:** +300–500 kcal surplus. Protein: 1.5–2.0 g/lb body weight minimum. Carbohydrates: primary energy source around workouts. Fats: 20–25% of calories. Water: 3–4 litres/day.

| Supplement             | Dose            | Purpose                                |
|------------------------|-----------------|----------------------------------------|
| TUDCA                  | 500 mg/day      | Liver protection (essential with Dbol) |
| Milk Thistle           | 600 mg/day      | Hepatoprotective antioxidant           |
| Omega-3                | 4 g/day         | Cardiovascular & lipid management      |
| Creatine               | 5 g/day         | Strength + recovery                    |
| Vitamin D3+K2          | 5000 IU+100 mcg | Testosterone production & bone health  |
| ZMA                    | Nightly         | Sleep quality + hormonal support       |
| Blood Pressure Monitor | —               | Check BP twice weekly throughout cycle |

■ *Educational purposes only. Always consult a licensed physician.*